# FOLFIRINOX followed by chemoradiotherapy · rectal cancer

This is a planned course before surgery assessment for locally advanced rectal cancer. Six FOLFIRINOX cycles are followed by capecitabine with pelvic radiotherapy. The team reviews response and the next part of your overall rectal-cancer plan.

**Locally advanced rectal cancer** · NCCP 00691 v4 · Document revised 4 November 2026

**Your personal plan:** this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

## My overall treatment

Phase 1: six FOLFIRINOX cycles, one every 14 days, each with a 46-hour fluorouracil pump. Phase 2: capecitabine twice daily on Days 1–5 of each seven-day week for five weeks, with radiotherapy. Your team supplies the separate radiation calendar.

**Phase 1 · six FOLFIRINOX cycles**
FOLFIRINOX every 14 days for six cycles; each includes a 46-hour fluorouracil pump.

**Phase 2 · five weeks with radiotherapy**
Capecitabine twice daily on Days 1–5 of each seven-day week for five weeks, with radiotherapy. The radiation unit provides the appointment calendar; the team confirms any treatment-day changes.

## Two phases with separate calendars

| Phase 1 · cycles 1–6 | Phase 2 · weeks 1–5 | After the two phases |
|---|---|---|
| Every 14 days. Day 1 infusions then 46-hour pump. | Capecitabine twice daily Days 1–5 of each seven-day week with radiotherapy. | Team-directed restaging and surgery/next-treatment assessment. |

Your team confirms the active phase and dates. Symptoms may occur during or after treatment; there are no guaranteed safe days.

## My tests and checks

| When | What is checked | Timing / reason |
|---|---|---|
| **Before** | **DPD / baseline bloods**<br>Required by regimen | DPD before fluoropyrimidine; blood count, kidney and liver tests. |
| **During** | **Bloods / nerves**<br>Required by regimen | Before each 14-day induction cycle and each weekly capecitabine cycle. Nerve review each induction cycle. |
| **If indicated** | **Heart**<br>Conditional tests | ECG if cardiac function is compromised. |
| **After / review** | **Recovery / scans / markers**<br>Team-agreed plan | Follow the individual cancer plan and any unresolved effects; scan and marker intervals are team-agreed. |

## Between treatments

- No guaranteed safe days: report fever, sudden illness, diarrhoea or poor intake promptly.
- Symptoms can continue after pump removal and after treatment ends.

Confirm supportive medicines, tests, working urgent contacts and pump disconnection.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**



Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

# What to watch for

You do not need to know which medicine caused a symptom before calling.

### 1 Mouth / swallowing

**Fluorouracil + folinic acid / later capecitabine**

Report ulcers or pain that affects eating or drinking.

### 3 Bowel / stoma

**Irinotecan / fluorouracil / radiotherapy**

Call early for diarrhoea, increased stoma output or poor intake; severe pain needs urgent assessment.

### 5 Blood / infection

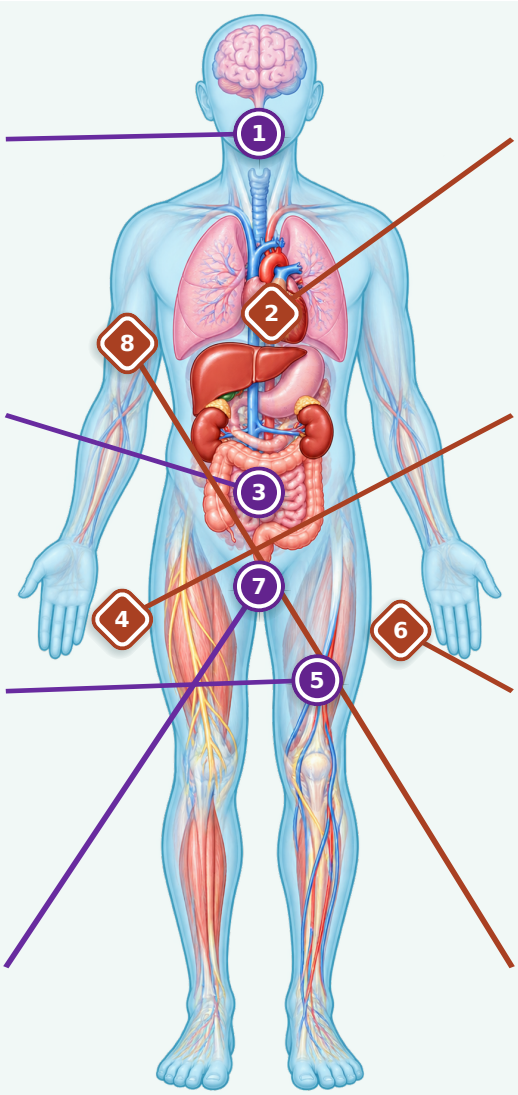
**Chemotherapy**

Fever, shivers or sudden illness: call now. Unusual bruising or marked weakness also needs advice.

### 7 Pelvis / bladder

**Pelvic radiotherapy**

Report painful urination, worsening pelvic symptoms or sore treatment-area skin.




### 2 Chest

**Fluorouracil / capecitabine**

New chest pain, collapse or major breathing difficulty needs emergency help.

### 4 Nerves / cold

**Oxaliplatin**

Avoid cold triggers; report persistent numbness or difficulty with buttons or walking.

### 6 Hands / feet

**Capecitabine / fluorouracil**

Painful redness or peeling affecting activity: hold capecitabine and call during the tablet phase.

### 8 Pump / line

**Treatment delivery**

Call for leakage, swelling, line pain or a loose connection. Do not reconnect it yourself.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or sudden illness needs immediate oncology advice. Chest pain, collapse or severe breathing difficulty needs emergency help. Call urgently for severe or persistent diarrhoea, severe abdominal pain, inability to drink, reduced urine or a line/pump problem.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

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# Keep your treatment details to hand

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Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

**FRONT / CONTACTS**

## My Treatment Card

SACTCheck

Name: _____

**FOLFIRINOX → rectal chemoradiotherapy**

6 × 14-day FOLFIRINOX (46-hour pump), then 5 weekly capecitabine + radiotherapy cycles.

Phase: FOLFIRINOX induction · 6 cycles

**Daytime (EXAMPLE)** 087 12345678

**Out of hours (EXAMPLE)** 087 87654321

**Unwell or fever? Call oncology now.**
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

**BACK / TREATMENT RECORD**

## FOLFIRINOX cycle record

Stamp the six induction cycles only.

| C1 | C2 | C3 |
|---|---|---|
| Date / initials | Date / initials | Date / initials |
| C4 | C5 | C6 |
| Date / initials | Date / initials | Date / initials |

My treatment information

Plan changed? Ask the team to update this card.

CUT outer border

**FOLD centre — do not cut**

## Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

## Chemoradiation record

Separate phase: team records each completed capecitabine/radiotherapy week. Follow the prescribed radiation calendar.

Week 1 _____ 2 _____ 3 _____ 4 _____ 5 _____ (date / initials)

## Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

### iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

### Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

## Your local contacts and resources

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.



Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.